

TABLE 14 Hierarchical rankings of first and second-line treatments recommended for management of acute bipolar I depression

	Level of evidence by phase of treatment					Considerations for treatment selection				
	Maintenance				Acute mania	Acute		Maintenance		Risk of manic/ hypomanic switch
	Acute depression	Prevention of any mood episode	Prevention of depression	Prevention of mania		Safety concerns	Tolerability concerns	Safety concerns	Tolerability concerns	
First-line treatments										
Quetiapine						+	++	++	++	-
Lurasidone + Li/DVP					n.d.	+	++	++ ^d	++/+	-
Lithium						+	+	++	++	-
Lamotrigine						++	-	-	-	-
Lurasidone					n.d.	-	+	-	+	-
Lamotrigine (adj)						++	+	++	++	-
Second-line treatments										
Divalproex						-	+	++ ^d	+	-
SSRIs/bupropion (adj)		n.d.		n.d.	n.d.	-	+	-	+	+
ECT						+	++	+	++	-
Cariprazine		n.d.	n.d.	n.d.		-	+	-	-	-
Olanzapine-fluoxetine		n.d.	n.d.	n.d.	n.d.	+	++	+++	+	+

adj, adjunctive; DVP, divalproex; ECT, electroconvulsive therapy; Li, lithium; SSRIs, selective serotonin reuptake inhibitors.

●, level 1 evidence; ●, level 2 evidence; ●, level 3 evidence; ●, level 4 evidence; ■, level 1 negative evidence; ■, level 2 negative evidence; ■, level 3 negative evidence; ■, level 4 negative evidence; n.d., no data; -, limited impact on treatment selection; +, minor impact on treatment selection; ++, moderate impact on treatment selection; +++, significant impact on treatment selection.

^aTrend for superiority on the primary efficacy measure, hence the lower rating.

^bEffective in those with an index episode of depression.

^cNegative data from the trial are probably due to methodological issues; rating based on expert opinion.

^dDivalproex and carbamazepine should be used with caution in women of child bearing age.

[Colour table can be viewed at wileyonlinelibrary.com]